

FREE GUIDE · BETTERHEALTH AFRICA

Your Family Health Map

A worksheet to map what runs in your family, the tests clinicians usually start with for each condition, and five questions to ask an older relative this weekend.

2-page PDF

Written by the BetterHealth Africa editorial team. Reviewed against WHO / ADA / AHA thresholds.

Why family history matters

Many of the conditions that are common in Ghana run in families: high blood pressure, diabetes, kidney disease, heart disease and stroke, sickle cell, gout, thyroid trouble and prostate cancer. Sharing a family does not mean sharing a diagnosis. It means some numbers are worth checking earlier and more often, while there is still plenty of time to act.

Knowing what runs in your family turns a vague worry into a short, specific list of tests, and a conversation with a clinician about when to start.

A family history raises the chance of a condition. It does not decide it. It does change the timing: screen earlier and more often, guided by a doctor, and use the results to make a better health decision.

Your family map

Tick, or write a word, for what you know. 'Not sure' is a fine answer; it tells you who to ask.

What runs in my family

Relative	High blood pressure	Diabetes	Stroke	Heart disease	Kidney disease	Sickle cell	Other
Mother							
Father							
Brothers and sisters							
Grandparents (mother's side)							
Grandparents (father's side)							
Aunts and uncles							

If this runs in your family, these are the tests clinicians usually start with

Family history and the first tests

Runs in the family	Usually checked first	BetterHealth panel
High blood pressure	A blood pressure reading (at least once a year for every adult), kidney function (creatinine and eGFR), blood sugar	Complete Health Check (Panorama); Heart Health Check (Cardion) for a focused look
Diabetes	HbA1c and fasting blood sugar, earlier and more often than the general guide	Blood Sugar Check (Dialics)
Stroke or heart disease	Lipid profile (cholesterol), blood pressure, blood sugar, and often CRP	Complete Health Check (Panorama) for the lipid profile; Heart Health Check (Cardion)
Kidney disease	Creatinine and eGFR, plus a urine test for protein	Core Health Check (Metabolix) or Complete Health Check (Panorama)
Sickle cell	A full blood count, and a haemoglobin electrophoresis test to identify your genotype	Full Blood Count (single test); ask about electrophoresis
Gout or kidney stones	Uric acid, read with kidney function	Complete Health Check (Panorama) or Heart Health Check (Cardion)

Runs in the family	Usually checked first	BetterHealth panel
Thyroid problems	TSH, usually with free T4	Women's Health Check (Empress) includes thyroid function
Prostate cancer (a father or brother)	A PSA conversation with a doctor from around age 40 to 45	Men's Health Check (Alpha)

As a general guide: ages 18 to 39 with no risk factors, a baseline screen then every two to three years; 40 and above, every one to two years; with a family history of diabetes, high blood pressure, kidney or heart disease, earlier and more often, guided by a doctor.

Why the same conditions keep appearing together

High blood pressure and diabetes are the two leading causes of kidney damage in Ghana. High blood sugar tends to raise triglycerides and lower HDL cholesterol. Gout shares risk factors with fatty liver and kidney trouble. So if one column on your map fills up, the tests in the neighbouring columns are usually worth doing at the same time. They share roots, and one panel often covers several.

One caveat if sickle cell is on your map

Around one in four Ghanaians carries the sickle cell trait, and it can make the HbA1c blood sugar test read falsely high or low. If sickle cell is on your map, tell the clinician reading your results. A fasting glucose test or an oral glucose tolerance test can stand in.

Five questions to ask an older relative this weekend

1. Has anyone in our family been told they have pressure (high blood pressure), sugar (diabetes) or kidney trouble? Who, and at roughly what age?
2. Did anyone have a stroke or a heart problem before 60?
3. Does anyone carry the sickle cell trait, or has anyone had sickle cell disease?
4. Has anyone had gout, thyroid trouble, or, for the men, prostate problems?
5. What did our grandparents pass away from, if you know?

Write the answers straight onto the map. You do not need the exact medical names. 'She was on tablets for pressure' is useful information.

What to do with a full map

- Bring the map to a clinician. It opens the conversation and shortens the visit.
- Book the tests in the table that match your fullest columns. One panel often covers several.
- Keep the map. Update it as you learn more, and share a copy with your brothers and sisters.

Sources

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